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Age, Affect, and Social Closeness: An Experience Sampling Study of Advice-Taking Across the Adult Lifespan

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Little is known about the degree to which older adults rely on advice in their everyday decision-making, as well as predictors and outcomes of their advice-taking. The present study aimed to examine whether social closeness and momentary affect predict advice-taking across the adult lifespan using an experience sampling method in everyday life. Decision satisfaction with or without advice-taking was also examined. Participants ($N = 117$) were predominately Western, European aged 21–76 years. They reported on their advice-taking 3 times a day for 10 days. Data collected over 2023–2024 revealed that degree of social closeness did not influence the association between age and advice-taking. However, when affect was more positive, older age was associated with greater advice-taking. These findings suggest that affective state but not relationship goals and closeness to an advisor predict greater advice-taking with older age. In addition, greater advice-taking and more positive affect are associated with greater decision satisfaction, regardless of age. Overall, the current findings indicate that in their everyday decision-making, older adults are more likely to take advice when they are in a positive mood.

Keywords: older adults, decision-making, advice, experience sampling, motivational goals

It is vital to understand how advice and emotion may influence the everyday decision-making of older adults. Not only can such research assist older adults in making decisions with the best possible outcomes, but it can also help practitioners and policy-makers understand when older adults may be willing to take recommendations, or when they may be more vulnerable to external and/or internal influences during decision-making (Mikels & Taullahu, 2023). Both important and everyday decisions involve taking the advice of others, either to diffuse responsibility for the decision outcome (Harvey & Fischer, 1997) or in an effort to improve accuracy (Rader et al., 2017). Motivation to diffuse responsibility might reflect an avoidant decision-making strategy, and decision avoidance has been suggested to increase in older age (for a review, see Mather, 2006), in turn suggesting greater advice-taking among older adults. Additionally, it has been proposed that older adulthood is a time in which there is a shift toward more emotionally meaningful goals, particularly with close social partners (Carstensen et al., 1999). A focus on emotional well-being via the maintenance of emotionally close social relationships may increase advice-taking when interacting with close advisors. The present study represented the first investigation of the association between age and advice-taking in everyday life using ecologically valid momentary assessment (Shiffman et al., 2008). It examines whether incidental affect and advisor closeness predict advice-taking across the adult lifespan, as well as age-related differences in decision satisfaction following advice-taking.

Decision-making processes appear to change with age. Relative to young adults (18–64 years), older adults (65–94 years) prefer to delegate decisions to avoid decision responsibility (Finucane et al., 2002). Older senior adults ($M_{age} = 77$ years, $SD = 3.0$) are also better than younger senior adults ($M_{age} = 67$ years, $SD = 3.5$) at learning to avoid negative outcomes (Frank & Kong, 2008), and preferences

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for social decision-making increase when decision-making abilities are perceived to be worse than same-aged peers (Smith et al., 2023). Although these avoidant decision-making strategies may not translate into a pattern of habitual decision avoidance (Leon et al., 2025), advice-taking may form part of older adults' arsenal of avoidant decision-making tactics, whether it is to avoid the responsibility of a decision or to avoid possible negative outcomes of a decision. Indeed, compared with young adults (18–37 years), older adults (62–84 years) have been found to take more advice (Bailey, Ebner, Moustafa, et al., 2021), noting that middle-aged adults were not included in this study. However, in comparison to young adults, their advice-taking was not as strongly influenced by the expertise of the advisor. Similarly, previous studies suggest that older adults are more trusting than young adults, even when there are indications of untrustworthiness (Bailey & Leon, 2019). In a further study involving the full adult age span, including middle-aged adults, there was no association between age and advice-taking even though older age was associated with rating advice as less accurate (Leon et al., 2024). Overall, younger adults appear to be more sensitive to sources of advice and are better able to adjust degree of advice-taking in response to perceived advice quality.

The tendency to take advice may be influenced by social closeness with the advisor or incidental emotions—that is, emotions that are unrelated to the decision (Loewenstein & Lerner, 2009). Socioemotional selectivity theory proposes that an age-related positivity effect, which is a cognitive processing preference for positive material relative to negative material, occurs as part of shifting motivational goals (Reed & Carstensen, 2012). A focus on knowledge acquisition and novel experiences when the future is perceived as long (as in young to middle adulthood) shifts to a focus on emotional gratification, well-being, and meaning in the present when time horizons are shorter (as in older adulthood). The latter goals are achieved in part via selective pruning of social networks to prioritize interactions with partners who satisfy emotional needs (Carstensen et al., 1999). This may help to explain why older adults involve a smaller social network than young adults when consulting with others on decisions (Wild & Löckenhoff, 2024).

Although older adults can override goals associated with the prioritization of relationship maintenance in favor of making a "good" decision (Wild & Löckenhoff, 2024), it is also possible that accepting advice is associated with social pressure to minimize feelings of rejection in the advisor (Harvey & Fischer, 1997). Advice-taking may therefore contribute toward maintaining social ties, particularly with those that are perceived to be socially closer (e.g., friends or family) than distant (e.g., a stranger). Consistent with this idea, people are more likely to take the recommendation of an advisor they have a close relationship with (Feng & MacGeorge, 2006; Schwartz et al., 2011), and this may be particularly prominent among older adults. Indeed, as noted, older adults prioritize maintaining relationships with close social partners, which may make them vulnerable to the wants of those partners (Löckenhoff, 2018). When in a situation involving close social contacts, emotionally meaningful interactions may be prioritized (English & Growney, 2021), and advice-taking may be a way to facilitate such interactions. With shorter time perspectives in older adulthood, there is a preference toward generative-related goals—that is, goals related to the well-being of others (Bailey, Ebner, & Stine-Morrow, 2021). In addition, among middle-aged and older adults (40–87 years), older age is associated with greater preference toward avoiding social conflict (Oberhauser et al., 2017). Together, this supports the idea that older decision makers may seek to avoid causing an advisor to experience negative emotion by accepting rather than rejecting their advice (Harvey & Fischer, 1997). Thus, older adults may be particularly motivated toward this type of socially involved decision-making.

Affect, encompassing both moods and emotions, is related to decision-making processes not only as a reaction to a decision outcome but also as part of feelings present during the decision situation (Loewenstein & Lerner, 2009). Lab-based research with young adults (undergraduate students) suggests that induced positive and negative affect influence advice-taking in different ways, with emotions such as anger being associated with less advice-taking, and emotions such as gratitude being associated with more advice-taking (de Hooge et al., 2014). Affect can therefore play a role in the extent to which someone takes the advice of another when they are faced with a decision. As noted, the age-related positivity effect is theorized to be driven by both shifting motivational goals, and the availability of cognitive resources, consistent with socioemotional selectivity theory (Reed & Carstensen, 2012). That is, both motivation and cognitive abilities contribute toward greater attention to, and memory for, positive relative to negative information in older age (Carstensen, 2006). This in turn enhances emotion regulation and positive affect among older adults (Erbey et al., 2020; Löckenhoff & Carstensen, 2004; Marsh & Crawford, 2024). Since greater cognitive ability has been associated with a stronger age-related positivity effect (Sakaki et al., 2019), it is possible that older adults in a less positive (and more negative) mood may possess lesser cognitive capacity and therefore may take more advice in order to decrease decision complexity (Bonaccio & Dalal, 2006). Previous lab-based research suggests that the positivity effect extends to older adults' (60–93 years) postdecision affect, in that they feel greater satisfaction in their choices than do young adults (18–39 years) (English & Carstensen, 2015; Kim et al., 2008; Löckenhoff & Carstensen, 2007). We therefore also expect older adults to report greater decision satisfaction regardless of the degree of advice-taking.

We included two measures of inductive reasoning (i.e., general fluid intelligence, Klauer et al., 2002). Inductive reasoning is engaged when using past experiences and/or current knowledge to predict future outcomes (Davidson, 2019; Hayes & Heit, 2018). It has been argued that inductive reasoning is also involved in identifying everyday domain-specific information patterns, such as those related to medication use, financial management, and food/nutrition preparation (Allaire & Marsiske, 1999). Furthermore, inductive reasoning difficulty among older adults has been reported to contribute toward poorer everyday problem-solving ability (Thornton & Dumke, 2005). Similarly, lower scores on fluid intelligence (measured via a progressive matrices task) among older adults have been associated with difficulty in discriminating the quality of advice (Bailey, Ebner, & Stine-Morrow, 2021).

The overall aim of the present study was to gain a better understanding of propensity to take advice in older age. We used experience sampling in everyday life to investigate advice-taking among young, middle-aged, and older adults. This methodology addresses a call to move beyond highly controlled experimental tasks (e.g., the judge-advisor task in Bailey, Ebner, Moustafa, et al., 2021; and Leon et al., 2024) when assessing older adults' social decision-making (Wild & Löckenhoff, 2024). In addition,

examining the full adult age span allows for a more nuanced understanding of the influence of age. We expected that with older age and greater levels of negative affect, there would be greater advice-taking (H1). The rationale for this hypothesis was that more negative affect among older adults may be associated with poorer cognition (Knight et al., 2007; Mather & Carstensen, 2005) and therefore leads to greater advice-taking in order to reduce the complexity of a decision (Bonaccio & Dalal, 2006). We therefore also explored correlations between advice-taking and inductive reasoning, noting that this indicator of general fluid intelligence typically suffers age-related decrements and is a core decision-making resource (Löckenhoff, 2018).

A secondary aim was to examine predictors and outcomes of advice-taking in older age. If advice-taking is a function of greater avoidant decision-making with age (Mather, 2006), then we expected a relationship between age and advice-taking that would be mediated by higher scores on an avoidant decision-making style questionnaire (H2). Drawing upon socioemotional selectivity theory and empirical data (Carstensen et al., 1999; Wild & Löckenhoff, 2024), advice-taking by older adults was expected to be greater when there was greater rated closeness to the advisor (H3). Last, momentary decision satisfaction was anticipated to be greater with older age (H4) in line with previous empirical studies (English & Carstensen, 2015; Kim et al., 2008; Löckenhoff & Carstensen, 2007). An exploratory analysis examined whether age and advice-taking predicted decision satisfaction.

Method

Transparency and Openness

We report how the sample size was determined, all data exclusions, and all measures in the study. The study was approved by the University of Technology Sydney Human Ethics Committee (ETH23-8268). The study design and analyses were preregistered prior to data collection via AsPredicted—study design at https://aspredicted.org/Y8V_C4P; all experience sampling hypotheses at https://aspredicted.org/6PP_YZR. Note that we also preregistered hypotheses and analyses specific to just the intake data, but these data are not reported within the current article. In addition, the first two of the six preregistered experience sampling hypotheses will be reported in a separate study. Thus, hypotheses 1, 2, 3, and 4 in the current article refer to preregistered hypotheses 3, 4, 5, and 6. All analyses were conducted in R-Studio (R Core Team, 2021; RStudio Team, 2020), and all R scripts, code and data are available at https://osf.io/gpjbn/. Note that to ensure participant anonymity, Prolific IDs and open responses have been excluded from the publicly available data file.

Participants

Kleiman Lab (2021) calculations indicated that a sample size of 120 participants is required to reach at least 80% power, with sampling three times a day for 10 days, with between 50% and 75% completion rate, and to detect a medium-sized effect of age on advice-taking. The default intra-class correlation coefficient of 0.5 was used for the calculation. With so few studies investigating age and advice-taking, it is difficult to estimate the expected effect size accurately. However, a large effect of age has been identified in at least one previous study (see Bailey, Ebner, Moustafa, et al., 2021). Although age was analyzed as a continuous variable, to ensure an even distribution of ages and gender across the adult lifespan, we aimed to recruit 40 young (18–39 years), 40 middle-aged (40–64 years), and 40 older (65+ years), with gender split evenly within each group.

Participants were recruited via prolific to participate fully online and were paid approximately £20 in total for the intake survey and experience sampling. Following an intake session, six young adults, 23 middle-aged adults, and 27 older adults withdrew from the study. The data for one young adult were excluded due to technical difficulties that resulted in the participant receiving too many surveys across the 10 days (65 surveys in total). The final data resulted in 39 young (M_{age} = 30.59, SD = 5.63, age range = 21–39), 40 middle-aged (M_{age} = 48.88, SD = 6.94, age range = 40–64), and 38 older adults (M_{age} = 68.66, SD = 3.16, age range = 65–76). The average compliance rates for completing surveys by each age group were 83.68 (SD = 16.5), 90.58 (SD = 13.39), and 86.75 (SD = 22.06) for young, middle-aged, and older adults, respectively. The gender, cultural background, and country of residence of each age group are shown in Table 1.

Materials and Procedure

Participants first completed an intake session where they provided informed consent and demographic information, and completed additional questionnaires and tasks. After completing these questionnaires and measures in Qualtrics (Qualtrics, Provo, Utah), participants were directed to a page with instructions on how to install the participation in everyday life application (Jessup et al., 2012) and control file to their devices. This page also included an information document for participants to download and refer back to if needed (https://osf.io/gpjbn/). Over the following 10 days, participants received notifications at three time points per day (between 9 am to 12 pm, 1 pm to 4 pm, and 5 pm to 9 pm) to complete a survey. Participants were allowed 60 min from the time of the notification to complete the survey, and reminder notifications were sent at 5 min and 15 min past the initial notification if the survey was not yet completed. On the 10th day, participants were provided with a debrief of the study and were paid for their time.

Avoidant Decision-Making Style

The avoidant subscale of the general decision-making style questionnaire (Scott & Bruce, 1995) was used to measure avoidant decision-making style. This subscale includes five items (e.g., "I generally make decisions at the last minute") rated on a 5-point response scale from 1 (*strongly disagree*) to 5 (*strongly agree*). The word "important" was removed from the original questionnaire statements, to keep them relevant to everyday, general decision-making. Cronbach's α for the present study was .86. Participants also completed the remaining subscales, but we do not analyze those responses in the present study.

Inductive Reasoning

Letter sets (Ekstrom et al., 1976) and number series (Thurstone, 1938) tasks were used to assess inductive reasoning as a marker of general fluid intelligence. We used shortened versions of each test,

Table 1
Demographics of Participants by Age Group

Demographic variable	Young 18–39 years (*n*)	Middle-aged 40–64 years (*n*)	Older 65–76 years (*n*)
Gender			
Woman	17	20	17
Man	21	19	20
Nonbinary	1	1	0
Undisclosed	0	0	1
Cultural background			
African	4	2	1
European	23	37	35
East Asian	1	0	0
South Asian	3	1	1
South-East Asian	7	1	0
Australian aboriginal	0	0	0
First nations or indigenous (unspecified)	0	0	0
Hispanic or Latino	2	0	1
Middle Eastern	1	0	0
Country of residence			
Australia	20	17	0
New Zealand	7	6	0
United Kingdom	10	16	24
United States	2	1	14

Note. For "cultural background," participants are instructed to select all options that applied to them. Only participants who completed both the intake and experience sampling are included. Although demographics are displayed as a function of age group, age was analyzed as a continuous variable.

adapted from Foster et al. (2015) and Hicks et al. (2016). In the letter sets, participants must select which letter set does not fit among four other options. Participants were first provided with one practice question and were then allowed a maximum of 5 min to complete 10 letter sets questions. For the number series, participants had to identify the correct number, out of five options, that would come next in a sequence. Participants were allowed 3 min and 30 s to answer 15 questions. Higher scores indicate better performance on each measure.

Experience Sampling Surveys

Surveys were administered via the free participation in everyday life application (Jessup et al., 2012), which allows for offline experience sampling. The first author created the control file for use within participation in everyday life (available at https://osf.io/gpjbn/). Participants were asked about their feelings in the past 3 hr on a scale from −3 (*very negative*) to +3 (*very positive*), the cause of these feelings (self-caused or caused by someone else), the nature of the last decision they made in the past 3 hr, and how important their decision was on a scale from −3 (*very unimportant*) to +3 (*very important*). They were then asked whether they had sought advice related to that decision (yes or no). If they answered "no" to this question, they were asked if they had received any advice (yes or no). If "yes" was answered to either seeking or receiving advice, they were asked if the advice had come from a person (including interactions that were online, via text messaging, or phone call) or nonperson (i.e., information available online, a recommendation that came from a website, and/or recommendations provided on a leaflet). When "person" was selected, they were asked to rate their closeness to this person on a scale of −3 (*not at all close*) to +3 (*extremely close*). Participants were told that an advisor rated as "not at all close" might be a person that they have never met before or do not have a strong connection to, while a person rated as "extremely close" might be a person that they know well, such as a very good friend, partner, or family member. For all participants who indicated seeking or receiving advice, they indicated the extent to which they had taken the advice from 0% (*completely ignored the advice*) to 100% (*followed the advice completely*). Finally, all participants indicated how satisfied they were with their decision on a scale from −3 (*very unsatisfied*) to +3 (*very satisfied*). The full survey questions and answer options can be found within the "Daily Surveys" codebook at https://osf.io/gpjbn/. The present study assessed questions relating to feelings (momentary affect), advice sought or received, closeness to advisor, proportion of advice-taking, and decision satisfaction. Additional questions unrelated to the current hypotheses were collected for a separate study.

Data Preparation

Outlier Data. Following our preregistered plan, and in line with our previous studies (e.g., Bailey, Ebner, Moustafa, et al., 2021; Leon et al., 2024), outlier data were identified as values more than +/−3 *SD* from the age group mean and were then adjusted to the age group's mean +/−3 *SD*. Among the young group, four momentary closeness to advisor ratings and four momentary advice taken ratings were adjusted. Among the middle-aged group, seven momentary advice taken ratings were adjusted. Among older adults, this resulted in the adjustment of seven momentary advice taken ratings.

Experience Sampling Exclusions. As per recommendations by Geeraerts and Kuppens (2020), experience sampling responses that were faster than 650 ms were identified as careless responses and were replaced with missing data. Across all the response variables, there were 33 instances of fast responding (0.01% of the full data set). There were no instances of surveys that had more than

50% of the items responded to faster than 650 ms. Following our preregistered plan, surveys that were sent outside of the specified schedule were removed. Eight participants received more than 30 surveys during the 10 days. Three of these participants (two older adults and one young adult) had restarted the survey schedule, resulting in 41, 39, and 31 surveys in total, and five other participants received an additional 1–6 surveys (for one young and two middle-aged adults). One older participant also appeared to receive surveys outside of the survey schedule, resulting in an additional 19 surveys received. The surveys that occurred beyond the bounds of the daily survey schedules were removed from the final data set.

Results

All analyses were conducted in R-Studio (R Core Team, 2021; RStudio Team, 2020).

Preliminary Results

Averaged across participants, advice was sought 644 times. On average, advice was sought 214.7, 251, and 178 times by young, middle-aged, and older participants, respectively. This sought advice was received on average 22%, 23%, and 18% of the time by young, middle-aged, and older participants, respectively. In addition, there were 649 instances of receiving advice from a person (young = 210; middle-aged = 244; older = 195), and 98 instances of receiving advice from a nonperson (young = 38; middle-aged = 39; older = 21). Additional descriptive information by age group is presented in Table 2, noting that the following preregistered analyses treat age as a continuous variable.

Intercorrelations

As per our preregistration, intercorrelation analyses were performed to investigate possible associations between age and the average proportion of momentary advice-taking in participants' everyday lives. The overall means of participants' momentary affect, decision satisfaction ratings, advisor closeness ratings, and inductive reasoning were also included, along with the avoidant decision-making style scores. Intercorrelations were calculated for all participants (with and without advice-taking) as well as for filtered data. Filtering resulted in the exclusion of 17 participants (eight young adults, three middle-aged adults, and six older adults) due to no instances of advice (seeking or receiving) throughout the sampling period, and/or due to missing data replacements for responses that were too fast (as described in the Method section).

Positive correlations were found between age and proportion of momentary advice-taking. The mean proportion of momentary advice-taking was positively correlated with mean momentary affect and mean decision satisfaction. Mean momentary affect was also found to be positively associated with mean advice quality ratings and mean decision satisfaction. The results are displayed in Table 3.

Mixed Effects Models

Where hypotheses were analyzed using mixed-effects models, level 1 predictors (within-person effects) were person-mean centered and level 2 predictors (between-person effects) were grand-mean centered. Random effects were selected based on the maximal model, as per recommendations by Barr et al. (2013). Fixed effects were tested simultaneously. Graphs were used to check assumptions, as per recommendations by Fife (2020). Where plots of residuals indicated heteroscedasticity, weighted least square models were created, as per recommendations by Rosopa et al. (2013), using the nlme package (José & Bates, 2024). Where the primary outcome variable was the extent of advice-taking, as indicated on a scale of 0%–100%, the percentage was divided by 100 so that this variable was between 0.0 and 1.0. Correlation analyses were performed using the Hmisc package (Harrell, 2020).

H1: Age, Affect, and Daily Advice-Taking

To assess the first preregistered hypothesis that older age and greater levels of negative affect would predict greater advice-taking, a model was created with age, between-person momentary affect, and their interactions as fixed effects, and within-person momentary affect as a covariate. The data were filtered to include only observations that included advice that was sought out or received. There were 762 instances of advice-taking (248 for young, 283 for middle-aged, and 216 for older adults). Random effects were attempted with intercepts and slopes for person-centered advice-taking, and person-centered affect, at the participant level. Variations were attempted until the maximal model converged. The random effects were intercepts and slopes for person-centered (within-person)

Table 2
Descriptive Statistics by Age Group

Variable	Young adults 18–39 years		Middle-aged adults 40–64 years		Older adults 65–76 years	
	M	*SD*	*M*	*SD*	*M*	*SD*
Inductive reasoning	5.97	1.83	5.72	1.40	5.62	1.52
Avoidant DMS	2.68	1.20	2.76	1.15	2.01	0.84
Average affect	1.19	1.45	0.96	1.47	1.22	1.46
Average decision impact	0.74	1.65	0.71	1.45	1.09	1.41
Average CTA	2.15	1.19	1.53	1.75	1.81	2.02
Average advice quality	1.99	1.04	1.97	1.12	2.16	1.12
Average advice-taking proportion	0.82	0.17	0.81	0.22	0.86	0.22
Average DS	1.58	1.22	1.57	1.22	1.89	1.19

Note. DMS = decision-making style; CTA = closeness to advisor; DS = decision satisfaction.

Table 3

Intercorrelations Between Age, and Participants' Average Momentary Affect, Average Closeness to Advisor Ratings, Avoidant Decision Style, Inductive Reasoning, Average Decision Satisfaction, and Average Momentary Advice-Taking

Variable	1	2	3	4	5	6	7
1. Age	—	.03		−.29**	−.04	.15	
2. Average momentary affect	.10	—		−.40***	.13	.79***	
3. Average advisor closeness	−.12	.13	—				
4. Avoidant decision-making style	−.34***	−.35***	.05	—	−.07	−.41***	
5. Inductive reasoning	.05	.08	−.03	−.08	—	.10	
6. Average decision satisfaction	.16	.79***	.10	−.36***	.13	—	
7. Average momentary advice-taking	.26**	.37***	.13	−.19	.08	.39***	—
Decisions without advice *M* (*SD*)	49.21 (16.44)	1.08 (0.86)		2.49 (1.12)	5.77 (1.59)	1.64 (0.66)	
Decisions with advice *M* (*SD*)	49.32 (16.20)	1.10 (0.85)	1.68 (1.29)	2.45 (1.12)	5.83 (1.48)	1.70 (0.59)	0.83 (0.13)

Note. Correlations for the filtered data (excluding participants who did not take any advice) appear below the diagonal. Correlations for the full data set (including participants who did not take any advice) appear above the diagonal.
** $p < .01$. *** $p < .001$.

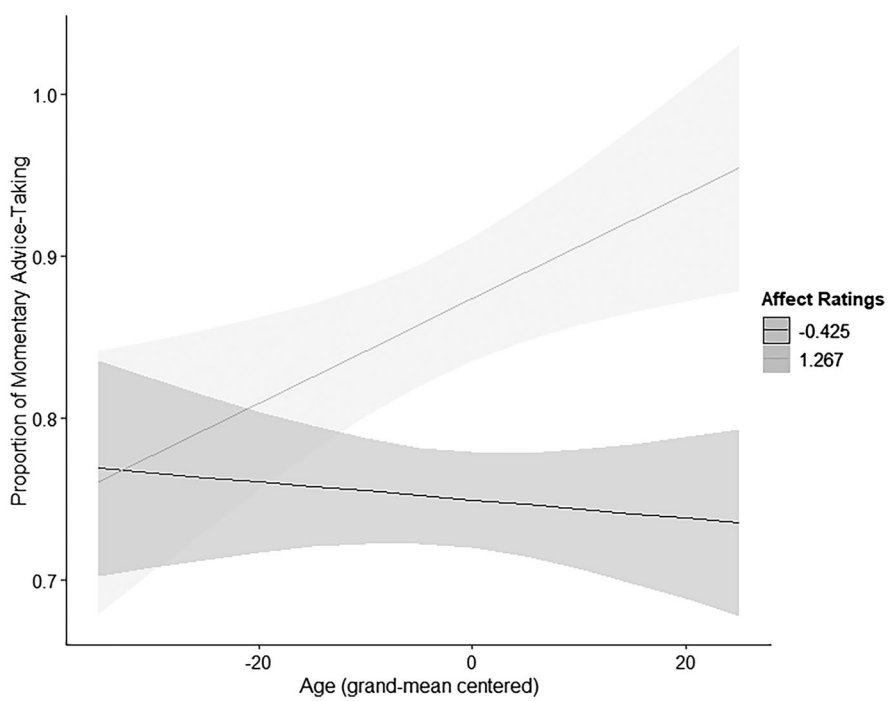
affect, and participant ID only. The final model was a weighted least square model with 746 observations and 100 participant IDs.

The results of the final model showed that age did not have a significant effect on the proportion of advice-taking in everyday life, $\beta = 0.0004$, $SE = 0.001$, 95% CI [−0.001, 0.002], $t = 0.5$, $p = .620$. Between-person affect ratings in everyday life were found to have a positive effect on advice-taking, such that with more positive affect, there was more advice-taking, $\beta = 0.073$, $SE = 0.014$, 95% CI [0.05, 0.10], $t = 5.4$, $p < .001$. Similarly, within-person affect ratings also had a positive effect on advice-taking, $\beta = 0.038$, $SD = 0.007$, 95% CI [0.02, 0.05], $t = 5.2$, $p < .001$. Furthermore, there was a small yet significant interaction effect between age and between-person affect ratings on advice-taking, $\beta = 0.002$, $SE = 0.001$, 95% CI [0.001, 0.004], $t = 2.7$, $p = .008$, as displayed in Figure 1. The proportion of variance explained by the fixed effects alone was 42%, while the proportion of variance explained by the entire model was 96%.

Simple slopes analyses revealed that with between-person affect ratings 1 *SD* above the mean, the relationship between age and momentary advice-taking is positive, $\beta = 0.003$, $SE = 0.001$, 95% CI [0.001, 0.006], $t = 2.7$, $p = .007$. With between-person affect ratings 1 *SD* below the mean, there is no relationship between age and momentary advice-taking, $\beta = -0.001$, $SE = 0.001$, 95% CI

Figure 1

The Interaction Between Age and Between-Person Momentary Affect Predicts Advice-Taking



Note. The affect ratings legend represents 1 *SD* below and 1 *SD* above, the mean between-person affect ratings. The *x* axis represents the continuous grand-mean centered age variable.

[−0.002, 0.001], $t = 0.6$, $p = .553$. As can be seen in Figure 1, when affect is more positive, older age is associated with greater advice-taking.

H2: Age, Avoidant Decision-Making Style, and Momentary Advice-Taking

The second preregistered hypothesis that a relationship between age and advice-taking would be mediated by avoidant decision-making style used mixed effects models with fixed effects of age, avoidant decision-making style scores, and their interaction. The data were filtered to include only observations that included advice that was sought out or received. The proportion of momentary advice-taking was set as the outcome variable, and random effects were attempted with intercepts and slopes for person-centered (within-person) advice-taking, at the participant level. Variations were attempted until the maximal model converged, and this included only the intercepts for participant ID. The final model was a weighted least squares model with 746 observations, and 100 participant IDs.

The model did not reveal any significant effects of age or the avoidant decision-making style, on the proportion of momentary advice taken, $\beta = 0.001$, $SE = 0.001$, 95% CI [−0.001, 0.002], $t = 1.7$, $p = .476$ (age), $\beta = -0.010$, $SE = 0.014$, 95% CI [−0.037, 0.017], $t = 0.7$, $p = .471$ (avoidant scores). Additionally, there was no significant interaction between age and avoidant decision-making style on momentary advice-taking, $\beta = 0.0001$, $SE = 0.001$, 95% CI [−0.001, 0.002], $t = 0.2$, $p = .866$. The model's fixed effects alone explained 3% of the variance, while the fixed and random effects of the model explained 92% of the variance. The hypothesis was not supported, and no further analyses were conducted.

H3: Age, Closeness to Advisor, and Momentary Advice-Taking

To assess the third preregistered hypothesis that with greater closeness to advisor ratings, there would be greater advice-taking among older adults, age, between-person ratings of closeness to the advisor and their interaction, were fixed effects in a mixed effects model. Within-person ratings of closeness to the advisor was a covariate. Data were filtered to include only those observations that included both advice-taking and ratings of advisor closeness. There were 210 instances for young adults, 244 for middle-aged adults, and 195 for older adults. Random effects were attempted with intercepts and slopes for within-person advice-taking, within-person ratings of closeness to the advisor, and participant ID. The maximal model included intercepts and slopes for within-person ratings of closeness to the advisor at the level of participant ID. The final model was a weighted least squares model with 649 observations and 97 participant IDs.

The model revealed a small significant positive effect of age on proportion of advice-taking, $\beta = 0.002$, $SE = 0.001$, 95% CI [0.0003, 0.0037], $t = 2.3$, $p = .024$. A positive effect of within-person closeness to advisor was also found, $\beta = 0.038$, $SE = 0.011$, 95% CI [0.01, 0.06], $t = 3.3$, $p = .001$. No effects were found for the between-person closeness to advisor ratings, $\beta = 0.022$, $SE = 0.011$, 95% CI [−0.003, 0.047), $t = 1.7$, $p = .090$, nor for the interaction between age and between-person closeness to advisor, $\beta = -0.002$, $SE = 0.001$, 95% CI [−0.0036, 0.0001], $t = 1.9$, $p = .058$. Thus, this hypothesis was not supported. The proportion of variance explained by the fixed predictors only was 19%, while the proportion of variance explained by the entire model was 97%.

H4: Age and Decision Satisfaction

The fourth preregistered hypothesis that greater rated closeness to the advisor would predict greater advice-taking among older adults was investigated using a mixed effects model, with the proportion of momentary decision satisfaction as the outcome variable, age as a fixed effect, and between-person momentary affect as a control. Instances of "N/A" coded decisions were filtered out from the full decision data (i.e., all decision observations were included regardless of advice being present). The maximal model included random intercepts and slopes for within-person proportion of momentary affect at the participant level. The final model was a weighted least squares model, with 2,897 observations and 117 participant IDs.

The model results supported our hypothesis, showing a significant positive effect of age on rated momentary decision satisfaction, $\beta = 0.007$, $SE = 0.002$, 95% CI [0.002, 0.012], $t = 3.0$, $p = .003$. There was also a significant positive effect of between-person momentary affect on decision satisfaction, $\beta = 0.584$, $SE = 0.044$, 95% CI [0.50, 0.67], $t = 13.2$, $p < .001$. The fixed effects of the model explained 35% of the variance, while both the fixed and random effects explained 54% of the variance.

Exploratory Analysis: Do Proportion of Advice-Taking and Affect Predict Older Adults' Decision Satisfaction?

An exploratory mixed-models analysis (not preregistered) was conducted to further investigate decision satisfaction. We sought to test whether greater advice-taking and more positive momentary affect predict greater satisfaction and whether this is related to age. The data were filtered to include only observations that included advice that was sought out or received. The proportion of momentary decision satisfaction was the outcome variable while age, between-person proportion of momentary advice-taking, and between-person momentary affect, as well as their interactions, were fixed effects. The maximal model included random intercepts and slopes for within-person proportion of momentary affect and within-person proportion of momentary advice-taking, at the participant level. The final model was a weighted least squares model with 746 observations and 100 participant IDs.

This model revealed no effect of age on rated momentary decision satisfaction, $\beta = 0.002$, $SE = 0.003$, 95% CI [−0.005, 0.009], $t = 0.6$, $p = .528$), but significant positive effects of between-person momentary advice-taking, $\beta = 2.328$, $SE = 0.438$, 95% CI [1.46, 3.20], $t = 5.3$, $p < .001$, and between-person momentary affect, $\beta = 0.256$, $SE = 0.068$, 95% CI [0.12, 0.39], $t = 3.8$, $p < .001$ on decision satisfaction. There were no significant interaction effects (all ps $> .05$). The fixed effects of the model explained 39% of the variance, while both the fixed and random effects explained 56% of the variance.

Discussion

The present study aimed to examine advice-taking in everyday decision-making across adulthood. We expected that older age and more negative affect would be associated with greater advice-taking

(H1). In direct contrast, when affect ratings were more positive, there was an association between older age and greater advice-taking. Contrary to our hypotheses, advice-taking with older age was not associated with a more avoidant decision-making style (H2), nor was closeness to the advisor associated with advice-taking in older age (H3). As expected, however, there was greater overall decision satisfaction with older age, averaged across decisions made with and without advice (H4). The exploratory analysis of decisions involving advice revealed that, averaged across age, both greater advice-taking and more positive momentary emotions are associated with greater decision satisfaction. Taken together, our results provide partial support for the suggestion that older adults' social decision-making is not necessarily driven by the maintenance of close social ties (Wild & Löckenhoff, 2024), but is associated more generally with positive affect.

The current research supports and extends Bailey, Ebner, Moustafa, et al.'s (2021) lab-based finding of greater advice-taking in older adulthood in particular circumstances, such as when an advisor is a novice. We show that in everyday life, older adults rely more on advice when their emotions are more positive. A previous lack of association between age and advice-taking may have been due to the constraints of the experimental task involving the estimation of the number of coins in a jar (Leon et al., 2024). In comparison, Bailey, Ebner, and Stine-Morrow (2021) used a judge-advisor task based on the estimation of rental property prices. The rental properties task may have been more reflective of real-world decisions, aligning with participants' everyday decision-making in the current research. Alternatively, age-related differences in advice-taking may be evident only under certain conditions, such as when an advisor is a novice or when the decision-maker is in a more positive mood.

Decision Style and Advisor Closeness Do Not Predict Advice-Taking

As noted, advice-taking may be considered a form of avoidant decision-making to the extent that it allows for the sharing of decision responsibility (Harvey & Fischer, 1997). Consistent with our meta-analysis (Leon et al., 2025), the present study revealed less avoidant decision style with older age, noting that the current decision style data contributed to that meta-analysis. However, there was no association between avoidant decision-making style and advice-taking. It is recommended that future research ask participants to rate the extent to which they did, or did not, want to make the current decision, as a more direct measure of decision avoidance.

In line with socioemotional selectivity theory and motivation to achieve emotional satisfaction in the present, social networks become smaller with age and meaningful interactions with people from these networks are prioritized by older adults (Carstensen et al., 1999; English & Growney, 2021). We therefore expected that with older age and greater closeness to an advisor, there would be greater advice-taking. Surprisingly, the data showed that, controlling for participants' within-person ratings of closeness to advisors, and averaged across age, advice-taking was not associated with social closeness to the advisor. This is inconsistent with previous research showing that people are more receptive to advice from closer social contacts (Feng & MacGeorge, 2006; Schwartz et al., 2011). However, the previous research focused on either recall of a personal situation in which participants received advice from another or involved fictious decision scenarios, rather than the everyday decisions and momentary advice-taking assessed in the present study.

It has been suggested that older adults prioritize positive emotions associated with making a good decision over goals associated with the maintenance of close social ties (Wild & Löckenhoff, 2024). It is therefore possible that regardless of closeness to an advisor, older adults perceive all social interactions as opportunities for experiencing positive feelings of emotional satisfaction and gratification. Given their life experience, older adults may also consider any advice (regardless of closeness) as an opportunity to improve decision accuracy. Further, older adults' advice-taking from others, regardless of closeness, may be a product of their preference toward goals associated with the well-being of others (i.e., generative-related goals) (Bailey, Ebner, & Stine-Morrow, 2021).

Affect Predicts Older Adults' Advice-Taking

Older age was associated with greater advice-taking in the present study when incidental affect ratings were more positive. In uncertain decision situations, such as those involving advice, a person's internal state (such as emotions) may be drawn upon to infer unknown information, such as an advisor's prior knowledge and reasons for behavior (Hawthorne-Madell & Goodman, 2019). As such, older adults in a positive mood may have attributed their emotions to the advice they encountered. In other words, they may have attributed the advisor's motives to be positive and/or the advisor's prior knowledge to be of good quality, resulting in greater advice-taking. It is possible that use of internal positive emotional cues to infer unknown information is an element of the age-related positivity effect. Specifically, focusing more upon positive material in older age (Carstensen & Mikels, 2005) may extend to internal information. Indeed, an internal positive cue (i.e., positive emotion) may have directed older adults' attention to positive information, and away from negative information, in their advice-taking decisions. This possibility should be tested in future research by directly comparing young and older adults' advice-taking while experiencing positive versus negative affect. It will also be important for future research to determine whether internal positive emotional cues result in older adults taking disadvantageous advice. To test this possibility studies must compare advice-taking in the context of advantageous versus disadvantageous decision outcomes. Indeed, it has been argued that relying on positive emotional cues may lead to older adults ignoring other relevant information when making a decision (Hanoch et al., 2007).

The age-related positivity effect and motivation to achieve emotional satisfaction in older age may be more evident among older adults with greater cognitive resources (Knight et al., 2007; Mather & Carstensen, 2005; Reed et al., 2014). We therefore expected that advice-taking would be greater among older adults in a negative mood potentially due to lesser cognitive capacity to devote to decision-making. Our data did not support this idea but are consistent with our previous research showing older adults with greater depressive symptoms did not engage in greater advice-taking (Leon et al., 2024). Additionally, our present results did not show any relationship between age and capacity for inductive reasoning, indicating that the current findings may not generalize to older adults who experience declines in this capacity. Future research may identify different associations between older adults' incidental emotion and advice-taking among samples who are less

cognitively high functioning, or by using different measures of cognition.

Advice-Taking Predicts Decision Satisfaction

The present results support previous lab-based studies showing greater decision satisfaction among older adults (60–93 years) relative to young adults (18–39 years; English & Carstensen, 2015; Kim et al., 2008; Löckenhoff & Carstensen, 2007). As hypothesised, our data indicated greater decision satisfaction with older age across all momentary decisions (i.e., with and without advice). Older adults' feelings of overall greater decision satisfaction likely contribute toward goals of emotional well-being and life satisfaction, as per socioemotional selectivity theory (Reed & Carstensen, 2012). However, when analyzing only decisions made with advice, there was no longer an association between age and decision satisfaction. Instead, when decisions incorporated advice, whether solicited or unsolicited, greater degree of advice-taking and more positive affect each predicted greater decision satisfaction averaged across the adult lifespan. Thus, regardless of age, people feel more satisfied with a decision that incorporates advice, perhaps because advice is known to improve decision accuracy (Bonaccio & Dalal, 2006) or enhances feelings of social connectedness (Fielder & Hütter, 2022). In addition, a positive mood may directly extend to greater feelings of decision satisfaction regardless of outcome.

Limitations and Future Directions

A clear strength of the present study is the use of ecological momentary assessment to analyze advice-taking in older adults' everyday lives. However, several limitations must also be acknowledged. For example, we did not measure perceptions of decision effort. Such a measure would enable comparison of advice-taking in the context of effortful versus less effortful decisions. This would provide insight as to whether older adults view advice as a way to reduce decision complexity (Bailey et al., 2023; Bonaccio & Dalal, 2006). We also did not measure emotions that were integral (directly related) as opposed to incidental (unrelated) to the decision at hand. There is some evidence that the two types of emotion may affect decision-making in different ways (for a review, see Mikels & Taullahu, 2023). Although a single-item measure of affect is a limitation of the present study, experience sampling studies require a trade-off between more comprehensive measures and maximizing participant compliance. Nevertheless, given that positivity was identified as a key predictor of older adults' advice-taking, future studies should consider including a multiitem measure of affect. We did not record the extent to which participants may have opportunities for social interactions in their everyday lives, and this should be considered for future research.

The role of cognition in decision-making may vary depending on the complexity of a decision. To gain a clearer picture of the potential influence of age-related differences in cognition on advice-taking, future experience sampling research should include a rating of how cognitively demanding participants found the decision. In addition, it is recommended that future research asks participants to rate the importance of meaningful interactions with each advisor, as well as the emotions associated with the interactions. This would allow for a more nuanced investigation of social information beyond closeness to the advisor that may be important in older adults' interactions with others and thus advice-taking. It would also be worthwhile to conduct follow-up studies to determine the extent to which older adults' advice-taking results in beneficial versus disadvantageous outcomes. This would help to more directly understand whether older adults' advice-taking leads to vulnerabilities in their decision-making.

Constraints on Generality

As noted, the current participants did not demonstrate age-related differences in capacity for inductive reasoning and therefore the findings may be specific to cognitively high functioning older adults. In addition, the study materials were in English, and the primary language of the authors was English, meaning that recruitment was constrained to English speaking participants. Therefore, most participants reported European cultural backgrounds. More culturally diverse participant samples are needed in future research to confirm the generalizability of the present findings. Future research should also capture participant socioeconomic status and education level. In addition, the present study was based on a cross-sectional sample of adults, and longitudinal studies are required to determine causal effects of age on advice-taking.

Conclusion

This study made a substantial contribution to the currently small body of research investigating advice-taking in older age. We provided evidence that in their everyday lives, advice-taking among older adults was not predicted by a more avoidant decision-making style, nor was it predicted by how socially close an older adult was to their advisor. Potentially the most important finding was that greater positive affect among older adults was associated with greater advice-taking. Older adults also reported greater decision satisfaction averaged across decisions with and without advice. However, in the context of decisions involving advice, greater advice-taking and more positive affect were associated with greater decision satisfaction averaged across the adult lifespan. Overall, the current findings indicate that older adults' social decision-making is not necessarily driven by the maintenance of close social ties (Wild & Löckenhoff, 2024), but is associated more generally with positive affect. The potential costs versus benefits of this behavior require further investigation.

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